

Experience Manager (EM)

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Role Definition & Clinic Process Document — Vetic

Version 1.0 | Prepared for clinic rollout & training

1. Purpose

The Experience Manager (EM) is a new clinic-floor role created to ensure every pet parent (PP) visit — OPD or grooming — is actively managed for experience quality, not just clinically/operationally completed. The EM is the dedicated human touchpoint responsible for proactive engagement, in-clinic issue resolution, and closing the feedback loop before the PP leaves the premises.

2. Role Overview

Role Title	Experience Manager (EM)
Base Location	Stationed at clinic — floor-based, mobile within clinic premises (reception, OPD area, grooming area, billing counter)
Reports To	Central CX team (functional — metrics, escalations, training) Be the point of human contact for the pet parent (PP) inside the clinic — proactively manage the visit experience end-to-end, resolve concerns on the spot wherever possible, and close the loop on feedback (record every interaction using google form; appts to interactions mapping and monitoring Rajat Tiwari).
Core Mandate	10am - 8pm
Shift Coverage	

3. Core Responsibilities by Journey Stage

VMS Pre-Check (Control Panel)	Before engaging the PP, check the VMS control panel (newly launched) for: customer profile, type of appointment PP has come in for, last feedback (happy/unhappy), and any active ticket. If the last feedback was unhappy or there is an active ticket, review the issue details and personally get involved in discussing and resolving it.
Arrival / Reception	Greet PP proactively (not reactively); set expectation on wait time; watch for visible distress (pet or parent) and flag to doctor/reception early.
Reception Loop Closure	Before PP exits, confirm: all queries answered, next steps / follow-up date understood, prescriptions or reports handed over, nothing pending. This is a mandatory checkpoint — no PP should leave with an open question.
Billing	Ensure PP is taken through itemized bill (consult fee, medicines, procedures, membership discounts) before payment; pre-empt billing disputes; confirm bill matches services actually rendered.
Grooming — Post	Conduct a post-grooming satisfaction check with PP before they leave — visually inspect pets with PP, ask direct satisfaction questions, not just "how was it."
Service Correction	If PP flags a grooming or service miss (uneven trim, missed service, wrong product used), trigger on-spot correction (re-groom, redo step, refund/waiver per authority matrix) rather than deferring to "next visit."
Concern Resolution	Own end-to-end resolution of any concern raised at the clinic — de-escalate, resolve within authority limit, or escalate per matrix below. Log every interaction regardless of outcome. (google form)
Feedback Collection	Actively solicit feedback before PP leaves premises — App/WhatsApp feedback.
Service Denials	If any service is denied, it is being reported - to be checked by EM

Note: Screen/Signage Check and Grooming — Pre (package/duration confirmation) are planned additions to this role but will be introduced in a later phase, once the current scope is stabilized.

4.. Detailed Process Flow

4.1 OPD Visit Flow

Before greeting → EM checks VMS control panel: customer profile, appointment type, last feedback (happy/unhappy), active tickets. If last feedback was unhappy or an active ticket exists, EM reviews the issue and personally leads the resolution conversation with the PP.

In case of OPD related issue raised (UH, esc ticket, denial etc) - handholding to be done; best provider to be given (as per SOP).

PP arrives → EM greets proactively, sets wait-time expectation. Engage with the pet - pat, give treat, make them comfortable (very very important).

At reception (loop closure) → EM confirms with PP: all questions answered, prescriptions/reports collected, follow-up date/instructions clear. This is a hard checkpoint — do not let PP proceed to billing with an open question.

At billing → EM to ensure PP is taken through the itemized bill line by line; addresses any confusion before payment, not after.

Exit → EM triggers feedback request (App notification / WhatsApp link) or captures a quick verbal reaction and logs it.

4.2 Grooming Visit Flow

Before starting service → EM checks VMS control panel: customer profile, appointment type, last feedback (happy/unhappy), active tickets. If the last feedback was unhappy or an active ticket exists, EM reviews the issue and personally gets involved in resolving it before/during the visit.

In case of grooming related issue raised (UH, esc ticket, denial etc) - handholding to be done; best provider to be given (as per SOP).

Post-service (satisfaction check) → EM physically reviews the groomed pet together with PP and asks a direct satisfaction question — this must happen before PP exits, not be assumed.

If PP is unhappy → EM triggers immediate service correction (re-groom / fix specific issue) within authority, or escalates per the matrix in Section 5.

Exit → feedback collection, same as OPD flow.

4.3 Continuous / Background Tasks (not visit-specific)

Case logging — every interaction (resolved or escalated) tagged and logged in the CX ticketing tool using the Function → Issue Type → Case Type taxonomy, for MoM tracking.

Keeping hygiene at the clinic in check; coordinate with CM, housekeeping and ensure all areas are clean and smell free (including guest washrooms)

5. Escalation & Resolution Authority Matrix

The EM is empowered to resolve the majority of in-clinic, experience-level issues directly. Clinical, financial (beyond a defined limit), and reputational-risk issues must be escalated — the EM's job in those cases is to stabilize and route quickly, not adjudicate.

Wait-time / communication gap	Yes — apologize, expedite, reset expectation	—
Grooming service miss	Yes — re-groom / redo on the spot	Clinic Manager
Minor billing discrepancy	Yes, within defined waiver limit	Clinic Manager
Doctor / clinical disagreement	Facilitate conversation only	Duty doctor / Clinical Lead — EM does not adjudicate clinical decisions
Membership mis-selling complaint	No	Clinic Manager + CX team (P0/P1 ticket) — verify what was communicated at point of sale
Suspected negligence / safety incident	No — stabilize situation, do not admit liability	Clinic Manager + Central CX immediately as P0
PP threatens legal / social media action	No	Central CX / Escalation desk immediately — do not negotiate independently
<i>Note: exact waiver limits and P0/P1 definitions should be finalized with Clinic Ops and slotted into this matrix before rollout.</i>		

6. Systems & Tool Access Required

The EM role is cross-functional by design (clinical visibility + billing + CRM + ticketing). The access list below should be provisioned before an EM's first shift.

VMS	View live appointment queue, doctor allocation, service status (OPD/grooming in-progress vs. done); use the newly launched control panel to check customer profile, appointment type, last feedback sentiment, and active ticket status before engaging PP	Read access, clinic-level (incl. control panel view)
EMR (Electronic Medical Records)	View case notes/diagnosis at a summary level	Read-only, restricted to active-visit records
Billing / POS module	Pull itemized bill, verify against services rendered, apply authorized waivers/discounts	Read + limited waiver-write access (capped)
CX Ticketing Dashboard	Log every concern (resolved or escalated) with tagging — Function → Issue Type → Case Type; raise P0/P1 tickets	Create + update ticket access
Tab/ Laptop	Check waiting-area screen mirroring is live; raise IT ticket if screens are down/showing wrong content	Raise-ticket access; view-only on display panel if available
Internal Directory / Escalation Contact List	Know who to call for clinical, billing, or safety escalations without delay	Standard

7. Daily Task Checklist

Before every visit

Start of shift

Every OPD visit

Every grooming visit

Before PP exits (every visit)

Mid-shift

Check VMS control panel — customer profile, appointment type, last feedback (happy/unhappy), active ticket status. If unhappy last feedback or active ticket, review details and personally get involved in resolution.

Check VMS queue for the day; review handoff notes from previous shift (open cases, pending escalations)

Greet → reception loop closure before exit → bill walkthrough

Post-service satisfaction check → on-spot correction if needed

Feedback collection (App/WhatsApp)

Review open tickets for status update

8. Success Metrics (KPIs)

Clinic-level NPS

UH% (Unhappy %) and on-spot resolution rate

Feedback capture rate

Post-grooming satisfaction check completion %

Repeat-complaint rate (same PP, same issue)

Overall experience sentiment for the clinic during EM's tenure/shift

Of the concerns raised, what % were resolved at the clinic itself vs. escalated

% of visits where feedback was actually collected (App/WhatsApp/verbal) — should trend toward 100%

Whether the post-grooming check is actually happening every time, not just on paper

Whether loop closure is genuinely closing loops or PPs are returning with the same issue

9. Reporting & Governance

Daily: Record every interaction, issue resolved etc using google form.

10. Training Requirements Before Go-Live

Systems training: VMS, EMR (read-only), billing/POS, Vetic App staff view, CX ticketing tool.

VMS control panel training (newly launched) — customer profile lookup, appointment type, feedback history, active ticket view.

Soft skills: de-escalation, active listening, explaining clinical/billing information in simple language.

Product knowledge: grooming packages, membership plans/benefits (critical for spotting and correctly handling mis-selling complaints), standard OPD process.

Escalation matrix walkthrough with real case examples (billing dispute, grooming redo, safety incident, mis-selling).

Shadowing: minimum 2–3 shifts shadowing an existing CX/clinic staff member before going solo.

11. Additional Recommendations

Visible identification: EM should wear a distinct badge/lanyard so PPs can identify who to approach — this is a proactive, not reactive, role and PPs should be able to spot the EM unprompted.

Data privacy: EM has visibility into medical records and billing — role must be covered under the same confidentiality/data-handling policy as clinical staff.

Feedback follow-up loop: detractor feedback (via App/WhatsApp) should route back to the EM or Clinic Manager for a follow-up contact within a defined TAT (e.g. 24–48 hrs), not just sit in a dashboard.

Avoid role overlap confusion: clearly communicate to reception and grooming staff that the EM is additive (owns experience/resolution) and does not replace their existing responsibilities — this should be included in the same training rollout for those teams.

Pilot before full rollout: consider piloting the role at 2–3 clinics of varying size/volume for 4–6 weeks, tracking the KPIs in Section 8, before scaling chain-wide.

Ongoing ticket or previous UH then to get best available service provider